

1. Administrative & Study Identification

Full Study Title: A Study to Assess the Efficacy and Safety of Induction Therapy With Afimkibart (Also Known as R07790121) in Participants With Moderately to Severely Active Ulcerative Colitis **Short Title/Acronym:** Ametrine-2 **Protocol Number:** GA45330 **NCT Number:** NCT06588855 **Sponsor Name:** Hoffmann-La Roche **Investigational Product:** Afimkibart (R07790121 / anti-TL1A monoclonal antibody) **Phase of Development:** Phase III **Medical Monitor:** Hoffmann-La Roche Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy and safety of induction therapy with Afimkibart (R07790121) compared with placebo in participants with moderately to severely active ulcerative colitis (UC). **Secondary Objectives:** To evaluate endoscopic improvement, histologic improvement, histologic-endoscopic mucosal improvement, and the overall safety, tolerability, and pharmacokinetic profile of Afimkibart.

2.2 Background & Rationale To address the high unmet medical need for treatments with better benefit-risk profiles in ulcerative colitis. While therapeutic options have expanded, a significant proportion of patients do not improve, lose response over time, or experience unacceptable adverse effects with current standard therapies (such as corticosteroids, biologics, and small molecules). Afimkibart is being investigated to attenuate inflammation and clinical sequelae, providing sustained control to improve the long-term prognosis of patients with moderately to severely active UC.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Placebo-controlled **Blinding:** Double-blind **Randomization:** Randomized (e.g., 3:2 ratio; 60% chance of receiving the study drug and 40% chance of receiving placebo)

3.2 Planned Enrollment & Duration Target \$N\$: 350 participants **Number of Sites:** ~198 locations globally **Estimated Study Start:** 11-Dec-2024 **Estimated Primary Completion:** 30-Jun-2027

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Confirmed diagnosis of moderately to severely active ulcerative colitis (UC) assessed by Modified Mayo Score (mMS) and confirmed by endoscopy. 2. Age 16 to 80 years old (including fully-developed 16 to 17-year-olds in select countries). 3. Body weight ≥ 40 kg. 4. History of inadequate response, loss of response, or intolerance to at least 1 other standard UC medicine.

4.2 Exclusion Criteria 1. Currently known complications of UC (e.g., fulminant colitis, toxic megacolon). 2. Current diagnosis of Crohn's disease (CD) or indeterminate colitis, microscopic colitis, ischemic colitis, infectious colitis, or radiation colitis. 3. Presence of an ostomy, ileoanal pouch, or current diagnosis/suspicion of primary sclerosing cholangitis. 4. Pregnancy or breastfeeding, or intention of becoming pregnant during the study. 5. Past or current evidence of definite low-grade or high-grade colonic dysplasia, adenomas, or neoplasia not completely removed.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: During the 12-week induction phase, patients receive treatment via infusion on Day 1, Week 2, and then every 4 weeks until Week 10. Participants completing induction may join an open-label extension (OLE) phase to receive injections every 2 to 4 weeks. **Route of Administration:** Intravenous (IV) followed by Subcutaneous (SC) injections. **Duration of Treatment:** 12 weeks of double-blind induction, followed by an optional open-label extension (OLE) phase, and an ultimate 12-week safety follow-up.

5.2 Concomitant & Prohibited Therapy Permitted: Stable doses of standard background UC therapies (e.g., oral aminosalicylates, systemic corticosteroids) as permitted per protocol parameters. **Prohibited:** Concurrent treatment with other biological agents, targeted synthetic therapies for IBD, or other investigational agents.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Endoscopy, Medical History, Baseline Assessments
Baseline	Day 1	Randomization, First IV Infusion Dose
Interim	Weeks 2, 6, 10	Onsite Visits: IV Infusion Doses, Efficacy and Safety Assessments
End of Induction	Week 12	Primary Endpoint Evaluation (Clinical Remission / Endoscopy), Entry into OLE

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Percentage of participants achieving Clinical Remission at Week 12. Clinical Remission is defined as achieving a Modified Mayo Score (mMS) ≤ 2 with a stool frequency subscore (SFS) = 0 or 1, rectal bleeding subscore = 0, and an endoscopy subscore ≤ 1 . **Measurement Method:** Clinical assessment using patient-reported symptoms and Central Endoscopy Reading.

7.2 Secondary & Exploratory Endpoints - Percentage of participants with endoscopic improvement. - Percentage of participants with histologic improvement and histologic-endoscopic mucosal improvement. - Reductions in rectal bleeding and stool frequency.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection via regular trial visits, laboratory tests, and immunogenicity (anti-drug antibody) tracking. **Serious Adverse Events (SAEs):** Evaluated continuously and required to be reported by the investigator within 24 hours of site awareness. **Data Monitoring Committee (DMC):** External, independent data monitoring committee to routinely assess unblinded safety data and ensure patient safety.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) Population for primary efficacy endpoints; Safety Set for all participants receiving at least one dose of the investigational product. **Sample Size Justification:** $N = 350$ participants, randomized in a 3:2 ratio, adequately powered to detect a statistically significant difference in Week 12 clinical remission between the Afimkibart and placebo groups. **Handling of Missing Data:** Non-responder imputation (NRI) for binary efficacy endpoints (e.g., subjects who withdraw prior to Week 12 or have missing data are considered non-responders).

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Standard onsite and remote clinical monitoring visits to ensure subject safety and protocol adherence. **Audit Trail:** Secure eCRF entry locking, central reading for endoscopic objectivity, and data clarification forms via standard quality audit mechanisms.

11. Study Identifiers & Governance

Primary ID: GA45330 **Registry Number:** NCT06588855 **Sponsor/Lead Organization:** Hoffmann-La Roche **Study Title:** Ametrine-2 **Official Regulatory Title:** A Phase III, Multicenter, Double-Blind, Placebo-Controlled Study to Assess the Efficacy and Safety of Induction Therapy With R07790121 in Patients With Moderately to Severely Active Ulcerative Colitis

12. Clinical Framework (Ulcerative Colitis Specific)

Primary Condition: Moderately to Severely Active Ulcerative Colitis **Diagnosis Threshold:** Modified Mayo Score (mMS) indicating moderate to severe disease, confirmed by Endoscopy **Medical Methodology:** Double-blind induction therapy targeting tumor necrosis factor-like ligand 1A (TL1A) **Optimization Target:** Clinical Remission and Endoscopic/Histologic Mucosal Improvement

13. Study Procedures & Methodology

Management Pathway: Induction therapy IV infusions transitioning to maintenance SC therapy via an Open-Label Extension **Education Protocol (HCP):** Site initiation visits, protocol training, and standardized central reader guidelines for endoscopies **Education Protocol (Patient):** Education on symptom tracking (ePROs) for stool frequency/rectal bleeding and training on SC self-injection for the OLE phase **Quality Audit Mechanism:** Centralized endoscopic reviews to eliminate observer bias, paired with standard clinical data auditing

14. Observation & Follow-Up Schedule

Total Duration: 12-week induction phase + optional open-label extension phase + 12-week safety follow-up period **Onsite Visit Cadence:** Day 1, Week 2, Week 6, Week 10, Week 12 (and regular intervals during the OLE phase) **Remote Monitoring Frequency:** Intermittent monitoring / electronic patient-reported outcome (ePRO) diaries **Checklist Review Interval:** Continuous tracking of stool frequency and rectal bleeding through patient diaries

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				